Challenge Stigma / Find Peace

FrontlineFolks | Guide #18

Substances as Off■Switches: Alcohol, Pills, and Numbing Out

A Field Guide for Frontline Folks and Their Families

Numbing tools work fast and cost long.

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Disclaimer

This guide is for general education only.

It is not:

- Therapy
- Medical care
- A crisis service
- Individualized advice

Reading this guide does not create a therapist–client relationship with the author or with Never Forgotten Services, PLLC.

If you are in crisis or thinking about harming yourself or someone else, call 988 in the United States, go to your nearest emergency room, or contact your local emergency services.

If you want mental health treatment, please contact a licensed provider in your state or call the number on the back of your insurance card.

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1. What Is Happening in Your System

Using substances to downshift after work is common in frontline culture. A few drinks to take the edge off, pills to sleep or manage pain, cannabis to quiet the brain—these work fast and feel like the only reliable off-switch. The problem isn't that they work; it's that they stop working and start costing. Tolerance builds, rebound effects worsen the problems you're trying to solve, and what started as a tool becomes a trap. This isn't moral failure—it's a nervous system trying to regulate with the fastest tool available.

Signs substance use is becoming a problem

- Increasing tolerance: Need more to get the same effect.
- Using daily or near-daily to manage stress, pain, or sleep.
- Rebound anxiety or agitation when not using.
- Sleep fragmentation: Substances help you fall asleep but wreck sleep quality.
- Morning regret, guilt, or promises to cut back that don't stick.
- Conflict with partners about use. Hiding or lying about amounts.

> **Not weakness:**
> Substance use as an off-switch is a symptom of a dysregulated nervous system. The fix isn't willpower—it's building alternative regulation tools.

2. Why It Gets Worse Under Frontline Load

Frontline work leaves you activated with no natural downshift. You come home wired, your nervous system is still scanning for threats, and nothing feels relaxing. Substances offer instant relief: alcohol suppresses arousal, benzos quiet anxiety, opioids numb physical and emotional pain, cannabis dulls hypervigilance. The problem is neurochemical: substances work by hijacking your brain's reward and regulation systems. Tolerance builds, rebound effects escalate (alcohol rebound anxiety, THC rebound irritability), and you need more to get less relief.

Typical amplifiers

- Post-shift activation with no natural downshift: Coming home wired with no off-switch.
- Loneliness and isolation: Using alone becomes a routine.
- Pain flares: Substances numb both physical and emotional pain.
- Environmental cues: Home, couch, end-of-shift become automatic triggers.

- Peer culture: Drinking or using is normalized and encouraged in some frontline communities.

> *Why this works: reducing total load and improving predictability lowers baseline arousal and restores decision quality.*

3. What To Do This Week

You can't replace an off-switch without building a new one. The goal isn't to white-knuckle sobriety—it's to give your nervous system alternative ways to downshift so substances aren't the only tool.

1. Delay first use by 30 minutes: Insert a buffer. Walk, breathe, shower, eat. Prove to yourself that automatic use isn't the only option.
2. Replace one use window with movement: Trade one drink/pill/hit for a 10-minute walk or cold shower. Build alternative downshift tools.
3. Remove high-risk stock at home: Don't keep large quantities. Remove convenience and automatic access.
4. Add one accountability check-in weekly: Call, text, or meet with someone who knows your pattern. Isolation enables escalation.
5. Track use for one week: Log every use (time, amount, trigger, outcome). Data shows you the pattern you can't see in the moment.
6. Seek formal support if use is escalating: Therapy, peer support, MAT (medication-assisted treatment) all work. You don't have to do this alone.

> Field rule: make the action small enough that you can still do it on a bad day.

4. For Partners and Family

Household stability improves when everyone knows the plan before stress spikes.

- Use specific observations, not labels: 'I've noticed you're drinking every night' not 'You're an alcoholic.'
- Set clear boundaries without shame: 'I won't cover for you at work' or 'I need you sober for this conversation.'
- Support alternatives: Encourage movement, connection, therapy—not just 'stop using.'
- Don't enable: Buying, hiding, or making excuses prolongs the problem.

- Get support for yourself: Al-Anon, therapy, or peer support for partners of people with substance issues.

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5. When To Escalate Care

Self-management is a starting point, not a lifetime sentence. Escalate when function, safety, or relationships keep declining.

- Use is escalating in frequency or amount.
- You're using to function at work or hide use from colleagues.
- Relationships, work performance, or safety are suffering.
- You've tried to cut back and can't sustain it alone.
- Withdrawal symptoms show up when you stop (shakes, sweats, panic, seizures).
- Suicidal thoughts or self-harm appear alongside use.

6. 7-Day Field Drill

Use the worksheet daily for one week. Measure trigger intensity, response size, and recovery time.

> **Daily drill:**
> Name one trigger, one body signal, and one reset action.
> At end of week, keep what worked and remove what did not.

Worksheet 1: Worksheet: Substances as Off■Switches: Alcohol, Pills, and Numbing Out

Use this to track substances used as an off-switch in real time and build a repeatable response plan.

Where did this pattern show up this week?

List specific moments at work, at home, or during transition.

What did it cost you?

Include effect on body, mood, decisions, and relationships.

What two actions will you run for the next 7 days?

Choose small, realistic actions you can perform under stress.

1. Trigger:

What event, cue, or context activated this pattern?

2. Early body signal:

What did your body do first?

3. Reset action used:

What 30-90 second action lowered intensity?

4. Better next response:

Write the exact behavior you will use next time.

> Track this daily for one week, then adjust based on what actually worked.

Worksheet 2: Worksheet: Substances as Off■Switches: Alcohol, Pills, and Numbing Out - Partner/Team Check-In

Use this page for one short, practical check-in each week.

What do others notice first when this pattern is active?

Ask for concrete observations, not labels.

What support helps most?

List what helps and what makes it worse.

What agreement will we practice this week?

One clear behavior from each side.

1. Signal phrase:

What phrase means pause and reset?

2. Time-to-return:

How long until re-engagement after a pause?

3. Repair step:

What specific repair happens if things go sideways?

> Keep check-ins short, specific, and repeatable.

Closing

You've seen and carried more than most people will ever know.

Blow ups and shutdowns are not proof that you're broken or that your relationship is doomed.

They are signs that your nervous system has been running in survival mode for a long time.

The fact that you picked up this guide and read it matters.

You are choosing to be intentional about how you show up for the people who matter to you.

That choice is a lot more powerful than having flawless days.

About the Author

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Michael is a U.S. Army combat veteran and licensed clinical social worker who specializes in working with military, veterans, and first responders—our frontline folks. After leaving the military, he spent years fighting his own demons and trying to figure out why he could hold it together on the job but feel completely wrecked at home. The turning point was realizing he wasn't the only one living that split life—and that nobody should have to feel that alone, confused, or broken by what the job asks of them.

Since then, his work has focused on trauma, the impact of high-stress jobs on home life, and helping families make sense of what the job brings home. He uses plain language, zero shame, and a "you're not crazy, this actually makes sense" approach to help frontline folks and their families understand what's happening in their nervous systems and learn how to move through the chaos together.